

Policy on Effective Communication with Patients and Families

«Hospital Name»

Document control

Document No.	HCO/PRE/POL/08	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» has a system for effective communication with patients and/or families, so that communication serves its purpose, meets patients where they are, and never crosses into disrespect.

1. Purpose

This policy sets out how «Hospital Name» communicates effectively with patients and families, identifies special situations needing enhanced communication, ensures that enhanced communication is done well, prevents unacceptable communication, and monitors how well all of this works.

This policy does not cover informed consent, patient education about healthcare needs, or complaint redressal — those are covered in other hospital policies.

2. Scope

This policy applies to all communication between staff and patients and/or families at «Hospital Name».

3. Policy standards

«Hospital Name» communicates with patients and/or families effectively; identifies special situations where enhanced communication is required; carries out enhanced communication

effectively for each identified special situation; and ensures there is no unacceptable communication.

«Hospital Name» monitors and reviews the implementation of effective communication.

4. Non-negotiable rules

- Do not communicate with a patient or family in a way that fails to serve its purpose — identify and work to overcome communication barriers.
- Do not leave special situations requiring enhanced communication unidentified.
- Do not leave an identified special situation without a detailed, trained approach to the enhanced communication it requires.
- Do not allow unacceptable communication — abusing patients, hurting religious or cultural sentiments, or communicating with disrespect.
- Do not leave the effectiveness of communication unmonitored and unreviewed.
- Staff who see a rule under this policy broken report it the same shift to the Quality Coordinator or the Medical Superintendent.

5. What we do

5.1 Effective communication

Communication with patients and/or families at «Hospital Name» is done effectively — serving its purpose. «Hospital Name» identifies potential communication barriers and has plans to overcome them — for example, a language barrier may be overcome through an interpreter. Any recognised model of effective communication may be adopted.

5.2 Identifying special situations

«Hospital Name» identifies special situations where enhanced communication with patients and/or families is required — for example breaking bad news, handling adverse events, handling an aggressive patient or family, talking to the family of a patient who has died, or counselling for a complicated intervention.

5.3 Delivering enhanced communication

For each special situation identified under Section 5.2, «Hospital Name» details the nature of the enhanced communication required — for example, one recognised model for delivering bad news is SPIKES (Setting, Perception, Invitation or information, Knowledge, Empathy, Summarize or strategize). Any named or documented model may be adopted for each situation.

5.4 No unacceptable communication

«Hospital Name» does not allow unacceptable communication — for example abusing patients, hurting religious or cultural sentiments, or communicating with disrespect. Anyone who witnesses such conduct stops the interaction, protects the patient, and reports it under Section 4.

5.5 Monitoring communication

«Hospital Name» has a system to monitor and review how effective communication is implemented — for example through feedback or complaint analysis from patients and other stakeholders.

6. Stop-work authority

Not applicable in the procedure-start sense. Section 5.4 prohibits unacceptable communication as a conduct obligation — anyone who witnesses such conduct stops the interaction, protects the patient, and reports it the same shift under Section 4. The Medical Superintendent acts on a confirmed breach as a conduct matter. This is not a procedure-start stop-work gate.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring effective communication across the organisation; acts on confirmed unacceptable-communication breaches; receives escalations under Section 4.
Quality Coordinator	Owns implementation of this policy; maintains the list of special situations; monitors communication effectiveness and reports findings.
Department Heads / Unit In-Charges	Train staff on effective and enhanced communication for their area; escalate unacceptable-communication incidents.
All Staff	Communicate effectively; recognise and report unacceptable communication the same shift.

8. Quality monitoring

Monitoring area	What is monitored
Effective communication	Training records at induction and periodically; barrier-identification and interpreter-arrangement records; observed-practice records for sampled cases.
Special situations identified	Written list of special situations; training pack covering that list; staff-awareness records for emergency, ICU, OT and ward staff.
Enhanced communication delivered	Detailed description of enhanced communication for each listed situation; records confirming enhanced communication was carried out for sampled cases; training records showing staff know

	the models in use.
No unacceptable communication	Conduct-incident records for confirmed breaches; action records by the Medical Superintendent; training records naming examples.
Monitoring and review	Observation or record-sample review; findings reported to the Medical Superintendent; action records when communication failures found.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

9. Training and staff acknowledgement

All staff covered by this policy are trained on effective and enhanced communication principles, including recognising and reporting unacceptable communication, at induction and periodically.

I have read the Policy on Effective Communication with Patients and Families of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to the Quality department, all department heads/unit in-charges, and all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
ICU	Intensive Care Unit
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre
SPIKES	Setting, Perception, Invitation or information, Knowledge, Empathy, Summarize or strategize (breaking-bad-news model)

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.8.a	Commitment*	Sections 3 and 5.1 address effective communication serving its purpose, identifying and overcoming communication barriers.
PRE.8.b	Commitment*	Sections 3 and 5.2 address identification of special situations where enhanced communication is required.
PRE.8.c	Commitment*	Sections 3 and 5.3 address detailing and delivering enhanced communication for each identified special situation, including the SPIKES model for bad news.
PRE.8.d	Commitment	Sections 3 and 5.4 address the prohibition on unacceptable communication and the conduct-reporting obligation when it occurs.
PRE.8.e	Achievement	Sections 3 and 5.5 address the system to monitor and review implementation of effective communication, with findings reported and acted on.

13. Required Records/Evidence Checklist

Effective communication — PRE.8.a (Commitment*)

- Training records on effective communication principles (induction and periodic).
- Barrier-identification and interpreter-arrangement records.
- Observed-practice records confirming effective communication in sampled cases.

Special situations identified — PRE.8.b (Commitment*)

- Written list of special situations requiring enhanced communication.
- Training pack covering that list, with staff-awareness records for emergency, ICU, OT and ward staff.

Enhanced communication delivered — PRE.8.c (Commitment*)

- Detailed description of enhanced communication for each listed special situation (including the adopted model for breaking bad news, for example SPIKES).
- Records confirming enhanced communication was carried out for sampled cases.
- Training records showing staff know the models in use.

No unacceptable communication — PRE.8.d (Commitment)

- Conduct-incident records for confirmed unacceptable-communication breaches.
- Action records by the Medical Superintendent for confirmed breaches.
- Training records naming examples so staff can recognise the line.

Monitoring and review — PRE.8.e (Achievement)

- Communication-monitoring review records (observations, record samples, or feedback/complaint analysis focused on communication).
- Findings reported to the Medical Superintendent.
- Action records when communication failures were identified.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.8.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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